Pedia - PITCH (EN)

PITCH.md - Pedia · Track 2 AI in Pediatrics · ANDCS × MIT Hackathon 2026

> **Event** - Hackathon ANDCS × MIT Hacking Medicine, 17–19 April 2026, Epitech Paris, Kremlin-Bicêtre **Track** - Track 2 · AI in Pediatrics **Format** - 3 min pitch + 2 min Q&A · slides only · no live demo **Language** - English (MIT judges) **Voice** - flowing prose, the cadence of our 90-second archived pitch (`archive/practice-pitch-1/myontologynote-pitch-90s.html`): principle opener · enumeration rhythm ("voice, text, photo") · declarative triads ("prescribed by the physician, paid by the medical structure. Parents always free") · cascading close ("a pediatrician pays two euros a day. Parents pay zero. Children gain years"). Seed / demo-day pitch discipline under the prose - Kevin Hale (YC), Michael Seibel (YC), Paul Graham. Series A deck guides referenced for structure only - our stage is pre-traction, our format is 3 min, not 30. Numbers over adjectives. Show don't tell. No fluff. **Demo** - still images (screenshots of `Pedia.html`), no interactive product. **Goal** - win Track 2.

Timing (3:00)

Time	Slide	Block	Speaker
0:00–0:20	**1**	Hook - problem	**Ayda** (medical credibility)
0:20–1:20	**2**	Anamnesis comparison - solution	**Fanette**
1:20–2:00	**3**	Product / POC - solution	**Fanette** (continuing)
2:00–2:45	**4**	Business · market, pricing, ask	**Fanette** *or* **Ayda** (decide at rehearsal)
2:45–3:00	**5**	Team · cascade close · thank you	same speaker as slide 4

Slide 1 - Hook (0:00–0:20) · **strong words, black background, giant type**

Three lines, massive typography, one thought per line. No bullets, no logo, no subtitle.

```
In pediatrics,
symptoms don't speak.
Parents do.
```

Second beat, same slide (click-fade reveals):

```
180 days lost between visits.
4 years to diagnose a rare disease.
```

Third beat (final reveal):

```
Pedia.
```

Script (Ayda, slow cadence) :

> "In pediatrics, everything relies on observation. A child doesn't describe their symptoms - their parents live them, every day.
>
> Ideally, a child is seen every three to six months. But between two consultations, one hundred and eighty days of observations escape the medical system. The result: a pediatric rare disease takes four years to be diagnosed.
>
> We are Pedia."

Source displayed small, bottom of slide: `[MIT]` for the 4-year figure.

Slide 2 - Problem → Solution · anamnesis comparison (0:20–1:20) · **the hero slide**

Split in half, same child, same appointment window, two renderings.

Left panel - **"What the doctor writes during the visit"**

Paper texture, handwritten serif, arrows, crossed-out words, fragments only.

```
Louis, 2y
→ sleep?
→ langage... retard
"ok au repas"
crèche ?
follow up 3mo
```

Right panel - "What Pedia hands the doctor before the visit"

Clean white, clinical typography, 15 structured rows from `ANAMNESIS_REPORT.md`.

Category	Pedia report
Chief complaint	Recurrent sleep-onset crises (14 episodes / 4 mo) · expressive-language stagnation (0 new word / 3 wk)
Personal history	Bronchiolitis at 8 mo · 3 otitis · no hospitalization
Family history	Maternal uncle - adult ASD diagnosis
Medications	None
Allergies	None declared · transient mild eczema at 14 mo
Nutrition	Growing food selectivity · refuses soft textures since W-8
Growth (weight/height)	50th percentile weight · 75th height · stable curves
Psychomotor development	Walked at 13 mo · first words at 18 mo · lexical plateau since W-12 · no symbolic play
Vaccinations	Up-to-date (2024 schedule)
Symptom - timing	Crises cluster 8–10 pm · 11 of 14 follow a daycare day
Symptom - intensity / tolerance	45–90 min sleep onset · inconsolable crying · 3–4 wakings per night, W-6 to W-1
Triggers / relievers	Worse after collective care · relieved by rhythmic rocking + dim light
Behavioral tolerance	Daytime fatigue, appetite ↓, variable eye contact
Associated signs	Object-lining play (12 mentions) · inappropriate laughter · hand stereotypies
Circle corroboration	Daycare educator independently reports same object-lining (4 mentions)

Script (Fanette, slows on the contrast) :

> "Same child. Same appointment. On the left: what the pediatrician can scribble during the visit. On the right: what Pedia assembled from forty-seven parental observations over four months.
>
> Our AI turns each observation into a standardized clinical indicator - frequency, intensity, duration, triggers - powered by the HPO ontology, seventeen thousand internationally standardized phenotypes.
>
> What the visit missed, Pedia rebuilt. That is the difference between an early CAMSP referral at two, and a diagnosis at seven."

Source tag: `[HPO - Monarch Initiative]`

Slide 3 - Product / POC (1:20–2:00) · **three interface screenshots, equal-weight grid**

Three screenshots side by side, captured from `Pedia.html`:

Screenshot	Source	Caption
A - Parent capture	`Pedia.html` screen-2	"Hold to speak. 30 seconds."
B - Live structuring	`Pedia.html` screen-3	"Transcription · HPO mapping · HDS hosting."
C - Doctor dashboard	`Pedia.html` screen-4	"Two-minute read. 47 observations, 5 weak signals."

All three in the same moment, showing the loop : parent speaks → Pedia structures → doctor reads.

Script (Fanette, continuing from slide 2) :

> "Pedia is a multimodal, multilingual pediatric journal, prescribed by the physician. Three stakeholders, three interfaces. Parents log every day, in their mother tongue: voice, text, photo. Physicians read a structured report in medical language before each appointment, with four-month evolution curves and the weak signals detected across all observations. Hospital services get longitudinal records and avoided reconvocations.
>
> Behind the scenes: transcription, clinical extraction, structuring into HPO. Hosted HDS, RGPD-compliant, nothing leaves the EU.
>
> If a concerning pattern appears, the physician is alerted first - never a diagnosis, only a pathway to explore. The clinical decision stays with the physician."

Slide 4 - Business (2:00–2:45) · **seed one-pager layout**

Single slide, five blocks, scannable in 10 seconds.

Header - one-line pitch

> **Pedia turns 180 lost days between pediatric consultations into a 2-minute clinical report.**

Block 1 - Market `[MIT]`

- **TAM France - €31M/yr** (10k pediatricians · 35k GPs seeing kids · 450 hospital services)
- **SAM - €9M/yr** (independent pediatricians + priority hospital services)
- **EU expansion** next on the HPO interoperability rail.

Block 2 - Pricing (**price < ROI on every tier**)

Parent	Independent pediatrician	Hospital service
Free, forever	**€49 / month**	**€12k / year**
-	Recovers 11 h / month of clinical time ≈ €1,058	Avoided reconvocations + T2A capture
-	**ROI ×21** · breakeven at 1m 30s saved total	**Payback < 6 months**

Block 3 - Unit-economics targets

CAC **< €300** · LTV **> €1,800** · **LTV / CAC > 6** · Churn **< 2%/mo** · GM **> 85%**

Block 4 - Where we are · next concrete step

- **Today**: prototype (`Pedia.html` mockup) · semiology validated by our two MDs.
- **Next**: 50-pediatrician chronometered pilot.
- **North-star KPI**: time saved per consultation ≥ 2 min. Every other KPI (CAC, LTV, churn, activation) flows from it.
- **Regulatory scope**: MDR class I, CE mark in scope.

Block 5 - Team + Ask

Six co-founders : Fanette, Elom (dev) · Raoul, Raphaël (dev + business) · Ayda, Anouar (MD - clinical credibility no tech team can replicate).

Pre-seed ask: €400k, funding three deliverables:

1. 50-pediatrician chronometered pilot
2. CE marking class I
3. Two paying hospital contracts

Script (Fanette or Ayda - decide at rehearsal) :

> "Three stakeholders. Prescribed by the physician, paid by the medical structure. Parents always free.
>
> An independent pediatrician pays forty-nine euros a month and recovers eleven hours of clinical time. ROI times twenty-one. A hospital service pays twelve thousand euros a year, payback under six months. On every tier, **price stays below the client's ROI**.
>
> TAM France: thirty-one million euros. Europe is the natural next step, on the HPO interoperability rail.
>
> Today we have a validated prototype. Our north-star KPI: at least two minutes saved per consultation, measured in a fifty-pediatrician chronometered pilot. We are raising four hundred thousand euros to run that pilot, get CE marking class one, and sign our first two hospital contracts."

Slide 5 - Team · cascade close · thank you (2:45–3:00) · **six faces, clean grid**

Layout : six square portraits in a single row (or 3×2 on narrow aspect ratios). Green accent `#6FB58A` for the strip under each photo. No extra text beyond name + role.

```
┌────────┬────────┬────────┬────────┬────────┬────────┐
│ Fanette│  Elom  │ Raoul  │Raphaël │  Ayda  │ Anouar │
│  dev   │  dev   │  dev + │  dev + │   MD   │   MD   │
│        │        │business│business│        │        │
└────────┴────────┴────────┴────────┴────────┴────────┘
```

Optional top-of-slide line: **"Two physicians. Four engineers. The pairing is the moat."**

Script (same speaker as slide 4, faces appear on screen) :

> "Two physicians. Four engineers. The pairing is the moat."
>
> *(beat, look at the jury)*
>
> "A pediatrician pays two euros a day, recovers eleven hours a month. Parents pay zero. Children gain years of avoided diagnostic wandering.
>
> Thank you."

Rehearsal notes:

- Slide 5 transitions in as the speaker says *"Two physicians. Four engineers."* - their own face is on the slide, direct eye contact with the jury.
- The cascade ("two euros a day... parents pay zero... children gain years") lands over the team grid still visible - the faces amplify the human-scale close.
- Hold the slide through the silence, then "Thank you."

Screenshots to produce from `Pedia.html`

All at 1440 × 900, PNG, full-bleed:

ID	Target slide	`Pedia.html` screen	Crop
A	Slide 3 left	screen-2	phone with voice CTA
B	Slide 3 middle	screen-3	voice capture + pipeline panel
C	Slide 3 right	screen-4	patient banner + narrative anamnesis + weak signals
D	Slide 2 right (optional overlay)	screen-4	narrative block alone, for the anamnesis report visual

Patient consistency : `Pedia.html` already shows **Louis M., 2 yrs**. No rename needed unless the team picks a different demo identity.

Team portraits for Slide 5

Six square head-and-shoulders photos, same framing, neutral background, green accent strip:

Who	Role	Filename (suggested)
Fanette	dev	`team/fanette.jpg`
Elom	dev	`team/elom.jpg`
Raoul	dev + business	`team/raoul.jpg`
Raphaël	dev + business	`team/raphael.jpg`
Ayda	MD	`team/ayda.jpg`
Anouar	MD	`team/anouar.jpg`

Shoot in the hackathon space Saturday afternoon if photos aren't already on hand. Crop square, ~600×600 minimum for Google Slides HD export. Consistent lighting across the six.

Pitch best-practice checklist (to cross before D-day)

> Anchored on Hale, Seibel, Graham - see *Sources* at the bottom of this doc.

- [ ] Slide 1 tells the judges what Pedia is within 20 seconds.
- [ ] Slide 2 **shows**, not tells - the jury sees the anamnesis gap, doesn't need to be convinced of it.
- [ ] Slide 3 is the product, not a concept drawing. Real screenshots only.
- [ ] Slide 4 has **numbers over adjectives**. No "best-in-class", no "revolutionary".
- [ ] Slide 5 has six aligned, consistent-lighting portraits; name + role visible on every tile.
- [ ] Every claim has a source tag visible on the slide: `[MIT]`, `[HPO]`.
- [ ] The ask is concrete: € amount + what it buys.
- [ ] Each slide defends a single idea. If it takes two claims, split it.
- [ ] Cascade close lands over the team grid, then silence, then "Thank you."

Rehearsal checklist

- [ ] Full run-through under 3:05, three times, on the clock.

- [ ] Ayda and Anouar validate the slide-2 right-column anamnesis rows against UE 5 pediatric semiology.
- [ ] Screenshots A, B, C produced and inserted at the intended crop.
- [ ] Q&A drill from `PRACTICE_QUESTIONS.md` - every answer under 20 s out loud.
- [ ] Who-speaks-what fixed (table at the top of this doc).
- [ ] No filler words ("so", "kind of", "basically") - record and cut.

Sources

Pitch craft (primary anchors, matched to our stage and format)

- Kevin Hale - *How to pitch your startup* · YC Startup School · https://www.ycombinator.com/library/6q-how-to-pitch-your-company
- Michael Seibel - *How to build the perfect demo day pitch* · YC · https://www.ycombinator.com/library/6s-how-to-build-the-perfect-demo-day-pitch
- Paul Graham - *How to convince investors* · http://paulgraham.com/convince.html

Series A deck guides (reference for structure only, not cadence)

- *How to build a great Series A pitch and deck* · YC Library · https://www.ycombinator.com/library/8d-how-to-build-a-great-series-a-pitch-and-deck

Content sources

- `[MIT]` - MIT Sloan Healthcare Initiative · MIT Media Lab Connected Health
- `[HPO]` - Human Phenotype Ontology, Monarch Initiative · https://hpo.jax.org

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